



PPG Meeting
31/07/2025 – Start Time 13:--

Attendees:

MN – Deputy Manager

SA – Practice Manager

MC, GM,

JB, BB, ZF, JS, SN, AW

Apologies:

CC, AK, LU, MAS, DR

- Review of last minutes of meeting – everyone agreed no changes were needed.
- Meeting started at 13:13
- Asked PPG members if they would like anything to be added on Agenda to discuss in todays meeting.
- Discussed 2025 national survey report for the Practice. Gave each patient a coloured copy of the report to review. Once they had reviewed results, they were then given a coloured copy of last years report to compare the scores.
- The report was explained by Practice Manager. This is a national survey sent out yearly to randomly selected patients each year and returned to the provider. The results are of those who have returned the survey. Once they compile the results along with all the other practices, they then publish the national data online and notify the Practices of their results.
- PM went through each question and compared the data with the PPG members. On each question asked the Practice improved the percentage compared to last year.
- PPG members were asked what the Practice could do to get more patients to engage with completing such surveys.
- When checking data online you can also check neighbouring practices, and we compared ours to our PCN practice and we could see a higher percentage of returned surveys for other practices.
- SN suggested we send out text messages when we know surveys will be sent out and if patients struggle to complete, we could ask them to bring into the Surgery to help.
- MN explained that as we are not the ones sending out the surveys, we wouldn't know which patients the survey gets sent too and this would mean we would have to send a text msg to all 6000 patients registered at the Practice.
- MN also highlighted that we do not want any influence on the answers provided and we would like honest feedback from the patients. The patients if they do come in for help may feel obliged to give better scoring than how they actually feel.
- Overall, we are satisfied the outcome of the survey results – we will try and educate the patients who attend the surgery and encourage them to complete such forms/surveys in the future.
- Also decided to display notices before they get sent out.

- Updated the members about not being able to prescribe Mounjaro and this needs to be done through weight management.
- BB asked when they will be able to book in for flu jab. MN explained Flu Season starts from October. MN also explained that the local pharmacies received the vaccines around 2-3 weeks before GP practices if they wish to have it sooner they can find out from their local chemist.
- The Practice has not yet received a confirmation date of delivery for flu jabs.
- Updates around the PCN clinics were shared.
- Meeting was kept brief due to summer holidays and many apologies.

Meeting adjourned 13:51



Next meeting: October 2025

MassHealth Pediatric Behavioral Health Medication Initiative

BACKGROUND

The Pediatric Behavioral Health Medication Initiative proactively requires prior authorization for pediatric members (generally members less than 18 years of age) for certain behavioral health medication classes and/or specific medication combinations (i.e., polypharmacy) that have limited evidence for safety and efficacy in the pediatric population.

As part of this initiative, the following situations will require a prior authorization:

1. **Behavioral health medication polypharmacy:** (i.e., alpha₂ agonists, antidepressants, antipsychotics, armodafinil, atomoxetine, benzodiazepines, buspirone, cerebral stimulants, donepezil, hypnotic agents, memantine, meprobamate, modafinil, mood stabilizers [agents considered to be used only for seizure diagnoses are not included], naltrexone, prazosin, viloxazine, and xanomeline/trospium) filled within a 45-day period for members less than 18 years of age:
 - Pharmacy claims for 4 or more behavioral health medications **if one of the following is included:** an antipsychotic, a benzodiazepine, divalproex/valproate, lithium, or a tricyclic antidepressant
 - Pharmacy claims for 5 or more behavioral health medications (regardless of the medications included)
2. **Antipsychotic polypharmacy:** overlapping pharmacy claims for two or more antipsychotics for at least 60 days within a 90-day period for members less than 18 years of age;
3. **Antidepressant polypharmacy:** overlapping pharmacy claims for two or more antidepressants for at least 60 days within a 90-day period for members less than 18 years of age;
4. **Cerebral stimulant polypharmacy:** overlapping pharmacy claims for two or more cerebral stimulants (immediate-release and extended-release formulations of the same chemical entity are counted as one) for at least 60 days within a 90-day period for members less than 18 years of age;
5. **Mood stabilizer polypharmacy:** overlapping pharmacy claims for three or more mood stabilizers (agents considered to be used only for seizure diagnoses are not included) for at least 60 days within a 90-day period for members less than 18 years of age;

- ~~6. **Benzodiazepine polypharmacy:** overlapping pharmacy claims for two or more benzodiazepines (hypnotic benzodiazepine agents, clobazam, nasal and rectal diazepam, and nasal midazolam are not included) for at least 60 days within a 90-day period for members less than 18 years of age;~~
- ~~7. **Antipsychotic** pharmacy claim for members less than ten years of age;~~
- ~~8. **Antidepressant, armodafinil, atomoxetine, benzodiazepine, buspirone, donepezil, hypnotic, memantine, meprobamate, modafinil, mood stabilizer (agents considered to be used only for seizure diagnoses are not included), naltrexone, prazosin, viloxazine, or xanomeline/trospium** pharmacy claim for members less than six years of age;~~
- ~~9. **Alpha₂ agonist or cerebral stimulant** pharmacy claim for members less than three years of age.~~

The reference table below lists the behavioral health medications included in the Pediatric Behavioral Health Medication Initiative. Further information on the prior authorization requirements, including approval criteria, can be found within the MassHealth Drug List at www.mass.gov/druglist.

Pediatric Behavioral Health Medication Initiative Medication List ¹							
Antidepressants		Mood Stabilizers					
amitriptyline	levomilnacipran	carbamazepine	excarbazepine				
amoxapine	mirtazapine	divalproex	pregabalin				
bupropion	nefazodone	gabapentin	topiramate				
citalopram	nortriptyline	lamotrigine	valproic acid				
clomipramine	paroxetine	lithium					
desipramine	phenelzine						
desvenlafaxine	protriptyline						
dextromethorphan/bupropion	selegiline ²						
doxepin	sertraline						
duloxetine	tranylcypromine						
escitalopram	trazodone						
esketamine	trimipramine						
fluoxetine	venlafaxine						
fluvoxamine	vilazodone						
imipramine	vortioxetine	Antianxiety Agents					
isocarboxazid	zuranolone						
Antipsychotics				alprazolam	diazepam ³		
				buspirone	lorazepam		
				chlordiazepoxide	meprobamate		
				chlordiazepoxide/ amitriptyline	midazolam ³		
				clonazepam	oxazepam		
				clorazepate			
				Hypnotics		daridorexant	quazepam
						doxepin ⁴	suvorexant
		estazolam	temazepam				
		eszopiclone	triazolam				
flurazepam	zaleplon						
lomborexant	zolpidem						
Alpha ₂ Agonists		clonidine	guanfacine				
		Stimulants				amphetamine	lisdexamfetamine
						dexmethylphenidate	methamphetamine
						dextroamphetamine	methylphenidate
				dextroamphetamine/ amphetamine	serdexmethylphenidate		
					dexmethylphenidate		
				Miscellaneous		armodafinil	modafinil
						atomoxetine	naltrexone ⁵
						donepezil	prazosin
						memantine	viloxazine

¹Short-acting intramuscular injectable and intravenous formulations are excluded from the Pediatric Behavioral Health Medication Initiative requirements.

²Emsam (selegiline) is the only selegiline formulation included in the Pediatric Behavioral Health Medication Initiative.

³Nasal and rectal diazepam and nasal midazolam formulations are excluded from the Pediatric Behavioral Health Medication Initiative requirements.

⁴Doxepin tablet is classified as a hypnotic agent and the Pediatric Behavioral Health Medication Initiative requirements for antidepressants do not apply. Pediatric Behavioral Health Medication Initiative requirements for hypnotics apply.

⁵Vivitrol (naltrexone injection) is excluded from the Pediatric Behavioral Health Medication Initiative requirements.